

Undergraduate Medicine

# Pediatrics Study Pack

CharaNas Medicine — Specialty Notes

Expanded notes with original schematic figures for revision. These are educational drawings inspired by standard undergraduate teaching themes, not copied textbook plates. Always follow your faculty curriculum and latest guidelines.

| Section | Focus                              |
|---------|------------------------------------|
| 1       | Development & assessment           |
| 2       | Fluid, fever & infection           |
| 3       | Respiratory & cardiac clues        |
| 4       | Safeguarding & common wards topics |
| 5       | Checklist & books                  |
| 6       | Quick pearls from these notes      |
| 7       | Recommended book sources           |

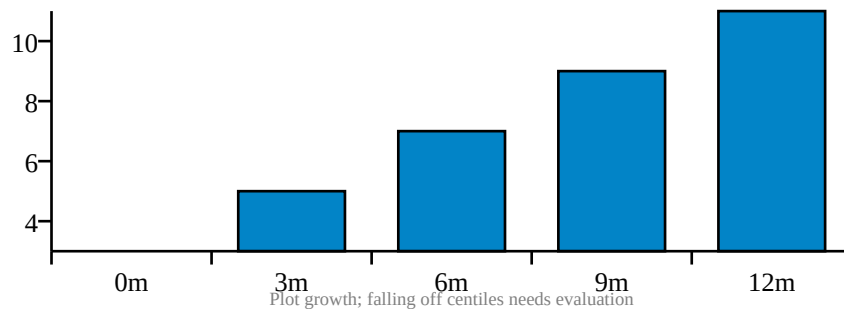
# 1. Development & assessment

Assess children by age: airway anatomy, fluid requirements, drug doses (usually mg/kg), and developmental milestones. Always calculate weight-based doses carefully.

Growth charts detect failure to thrive and chronic disease. Parental concern matters — listen carefully.

APGAR at 1 and 5 minutes summarizes early newborn transition, not long-term outcome alone.

Figure: Example weight-for-age idea (schematic chart)



Schematic figure for learning — verify details in your recommended textbooks.

## Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about pediatrics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

## 2. Fluid, fever & infection

ORS is first-line for most dehydrating diarrheas if the child can drink and is not in shock. IV fluids for severe dehydration/shock.

Fever in neonates is serious until proven otherwise. Immunization history changes differential and urgency.

Kawasaki disease: prolonged fever plus mucocutaneous signs — coronary aneurysm risk; early IVIG matters.

### Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about pediatrics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

### 3. Respiratory & cardiac clues

Respiratory distress: count RR, look for recessions, grunting, nasal flaring, SpO<sub>2</sub>. Bronchiolitis, croup, pneumonia, and asthma mimics are common.

Congenital heart disease may present with murmur, cyanosis, poor feeding, or shock in the ductal-dependent period — prostaglandin pathways in neonates are specialty-led emergencies.

Avoid over-oxygenation pitfalls in some congenital lesions — follow specialty guidance.

#### Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about pediatrics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

## 4. Safeguarding & common wards topics

Safeguarding: injury inconsistent with history, delayed presentation, patterned bruises — escalate via local child-protection pathways.

Vaccines: know live vaccines (e.g., MMR) vs inactivated; counsel parents on schedule and contraindications.

Common wards: bronchiolitis supportive care, asthma action plans, gastroenteritis hydration, neonatal jaundice assessment.

### Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about pediatrics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

## 5. Checklist & books

Checklist: age-specific normals, weight, hydration, work of breathing, rash, meningism, immunization status, safeguarding screen, caregiver understanding.

Books: Nelson Textbook of Pediatrics; Lissauer Illustrated Paediatrics; local IMCI/hospital protocols.

### Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about pediatrics.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

## 6. Quick pearls

High-yield reminders packaged with this specialty in CharaNas:

1. ORS first-line for most pediatric dehydration from diarrhea.
2. MMR is live; know live vs inactivated vaccines.
3. Kawasaki: prolonged fever + mucocutaneous signs → coronary risk.
4. APGAR at 1 and 5 minutes guides immediate newborn status.
5. Always calculate pediatric doses by weight; watch fluid rates.
  - Link symptoms → anatomy/physiology → investigation → first management step.
  - Write one OSCE-style explanation for a common presentation in this specialty.
  - After reading, do the Short MCQ and Case-based Question for active recall.

## 7. Recommended book sources

Standard undergraduate references commonly used internationally. Use the edition your college recommends. Figures in commercial textbooks are copyrighted; use library/licensed access for full plates and photographs.

| # | Reference                                      |
|---|------------------------------------------------|
| 1 | Nelson Textbook of Pediatrics                  |
| 2 | Illustrated Textbook of Paediatrics — Lissauer |
| 3 | Forfar and Arneil's Textbook of Pediatrics     |

How to study with this PDF: read one section → sketch the figure from memory → answer the MCQs/cases → review mistakes → revisit the matching section.

Disclaimer: educational aid only; not a substitute for clinical guidelines, faculty teaching, or licensed textbooks.